

SOUTH CAROLINA DEPARTMENT OF HEALTH AND HUMAN SERVICES

Long Term Living Physician's Input Letter

Community Choices, Nursing Home, or TEFRA Medicaid

FROM: _____

FAX #: _____

TO: _____

APPLICANT'S NAME: _____

SSN: XXX-XX-_____, DOB: _____

Please complete the area below and return to the FROM address above or FAX # above. This applicant is being evaluated for one of SCDHHS Community Long Term Care programs to help individuals, who want to live at home but need assistance with their care or who are seeking Medicaid-sponsored nursing home placement or TEFRA Medicaid (for children under age 19 with a childhood disability who can be provided care at home more economically than in a hospital or community home). To qualify, the applicant's Level of Care determination must be **Intermediate** or **Skilled**. This applicant's Level of Care cannot be determined by the information currently available and therefore we are requesting your input before making a final Level of Care determination. **Please forward a copy of the patient's History & Physical and the MOST current progress notes.** Please respond to the following questions. A signed Consent release form from the applicant/primary contact/legal guardian is enclosed.

Diagnoses:

Medications/Treatments:

Intermediate or Skilled Medical Needs/Monitoring: (Please see the attached Services List)

Do you feel this patient's **MEDICAL** condition is **STABLE** or **UNSTABLE**? Circle one.

If **UNSTABLE**, please specify reason and attach supporting documentation:

Physician Signature: _____ Date: _____

_____ Date: _____

CLTC Nurse Consultant

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Skilled Services

1. Daily monitoring/observation and assessment due to an unstable medical condition, which may include overall management and evaluation of a care, plan which changes daily or several times a week.
2. Administration of medications, which require frequent dosage adjustment, regulation, and monitoring.
3. Administration of parenteral medications and fluids, which require frequent dosage adjustment, regulation, and monitoring. (Routine injection(s) scheduled daily or less frequently [such as insulin injection] do not qualify.)
4. Special catheter care (e.g., frequent irrigation, irrigation with special medications, frequent catheterizations for specific problems.)
5. Treatment of extensive decubitus ulcers or other widespread skin disorder. (Important considerations include: Signs of infections, full thickness tissue loss, or requirement of sterile technique)
6. A single goal-directed rehabilitative service (speech, physical, or occupational therapy) by a therapist 5 days per week. Combinations of therapies will satisfy this requirement.
7. Time-limited, goal-directed, educational services provided by professional or technical personnel to teach self-maintenance, such as education for newly diagnosed or acute episodic conditions (e.g., medications, treatments, procedures).
8. Nasogastric tube or gastrostomy feedings.
9. Nasopharyngeal or tracheostomy aspirations or sterile tracheostomy care.
10. Administration of medical gases (e.g., oxygen) for the initial phase of condition requiring such treatment, monitoring, and evaluation (generally no longer than two-week duration).
11. Daily skilled monitoring or observation for conditions that do not ordinarily require skilled care, but because of the combination of conditions, may result in special medical complications. In these situations, the complications and the skilled services required must be documented.
12. This individual is totally dependent in all activities of daily living: incapable of locomotion; unable to transfer; totally incontinent of urinary or bowel function; must be totally bathed, dressed, and toileted and need extensive assistance to eat.

Intermediate Services

1. Daily monitoring of a significant medical condition requiring overall care planning in order to maintain optimum health status. The individual should manifest a documented need, which warrants such monitoring.
2. Supervision of moderate/severe memory, either long or short term, manifested by disorientation, bewilderment, and forgetfulness, which requires significant intervention in overall care planning.
3. Supervision of moderately impaired cognitive skills manifested by decisions, which may reasonably be expected to affect an individual's own safety.
4. Supervision of moderate problem behavior manifested by verbal abusiveness, physical abusiveness, or socially inappropriate/disruptive behavior.

Functional Deficits

1. Requires extensive assistance (hands-on) with dressing and toileting and eating and physical help in bathing. **(All four must be present and, together, they constitute one deficit)**
2. Requires extensive assistance (hands-on) with **locomotion**.
3. Requires extensive assistance (hands-on) with **transfer**.
4. Requires frequent (hands-on) with **bladder** or **bowel incontinent care**; or with **daily catheter** or **ostomy care**.